

Case N.H.: Writing the Plan

Read the case, then work through the 6 questions.
Choose your answer for each and be ready to explain your choice in class.

Card 1 · The case

N.H., 58-year-old woman

S: School administrator. Referred for BP review. "My ankles swell by the afternoon, and my shoes feel tight." Amlodipine 5 mg once daily for 1 year. No chest pain or breathlessness. Non-smoker. No known drug allergies.

O: BP 152/94 and 150/92 mmHg (two visits). Bilateral mild pitting ankle edema. LDL 170 mg/dL. 10-year ASCVD risk 12%. SCr 0.8 mg/dL, K⁺ 4.2 mmol/L. BMI 29 kg/m².

A: 1. Hypertension, above goal (goal less than 130/80 mmHg). 2. Ankle edema, likely amlodipine-related. 3. Elevated LDL with intermediate cardiovascular risk; not on a statin.

Swipe for the interview findings →

Card 1 · What the interview revealed

Goals

"I want to keep working until I retire, and not end up like my mother," who had a stroke at 64.

Preferences

"I prefer tablets in the morning only. I don't mind two if they're cheap."

Barriers

Pays for her medicines herself, so cost is her biggest concern. Last Ramadan she stopped her BP tablet for the whole month.

Support

Her husband reminds her about appointments.

Readiness

Walking after work: 7/10. Cutting salt: 3/10 ("my family won't eat bland food").

Card 2 · Choosing the therapy

Which option is best for N.H.'s blood pressure?

- A. Increase amlodipine to 10 mg once daily
- B. Continue amlodipine 5 mg and add generic lisinopril 10 mg once daily, both in the morning
- C. Switch to a brand single-pill combination (amlodipine/ARB) once daily
- D. Stop amlodipine and start hydrochlorothiazide 25 mg once daily

Card 3 · Testing in the plan

N.H. starts lisinopril 10 mg daily. Which monitoring plan is most appropriate?

- A.** Lipid panel in 1 week
- B.** Serum potassium and creatinine in 1–2 weeks
- C.** No laboratory tests; recheck BP in 6 months
- D.** A1c in 3 months

Card 4 · A complete plan entry

Which is the best-written plan entry for N.H.'s hypertension?

- A.** Change BP medication
- B.** Add lisinopril
- C.** Add lisinopril 10 mg once daily in the morning; check K^+ and SCr in 1–2 weeks; counsel on dry cough and lip swelling; review BP and edema in 4 weeks
- D.** Consider an ACE inhibitor

Card 5 · Planning for Ramadan

Last Ramadan, N.H. stopped her BP tablet for the whole month. What is the best plan?

- A.** Advise her not to fast
- B.** Before Ramadan, agree that she takes both once-daily tablets at the same time each day, at suhoor or iftar
- C.** Tell her to stop both tablets during Ramadan and restart afterwards
- D.** Switch to a three-times-daily regimen

Card 6 · Traceability

N.H.'s plan includes the items below. Which shows a discrepancy?

- A.** Add lisinopril 10 mg daily
- B.** Start atorvastatin 20 mg daily
- C.** Start omeprazole 20 mg daily
- D.** Refer to a dietitian

Card 7 · SMART goals

N.H. has just started lisinopril 10 mg once daily. Which adherence goal, agreed with her, meets all SMART criteria?

- A.** N.H. will take her lisinopril regularly to control her blood pressure.
- B.** N.H. will take lisinopril 10 mg every morning with breakfast on 7 of 7 days, starting today, and review this at her 4-week visit.
- C.** N.H. will take lisinopril 10 mg every morning without missing a single dose, for life.
- D.** N.H. will take lisinopril 10 mg every morning with breakfast.